Plendil® 2.5 mg, 5 mg and 10 mg

felodipine

Tablets

Composition
Felodipine 2.5 mg, 5 mg and 10 mg

Pharmaceutical form
Film-coated extended-release tablets based on the hydrophilic gel matrix principle.

The Plendil 2.5 mg tablet is yellow, circular, biconvex, engraved A/FL on one side and 2.5 on the other side, with a diameter of 8.5 mm.

The Plendil 5 mg tablet is pink, circular, biconvex, engraved A/Fm on one side and 5 on the other side, with a diameter of 9 mm.

The Plendil 10 mg tablet is reddish-brown, circular, biconvex, engraved A/FE on one side and 10 on the other side, with a diameter of 9 mm.

Therapeutic Indications
Hypertension
Stable angina pectoris

Posology and method of administration
The tablets should be taken in the morning, be swallowed with water and must not be divided, crushed or chewed. The tablets can be administered without food or following a light meal not rich in fat or carbohydrate.

Hypertension
The dose should be adjusted individually. Treatment should be started with 5 mg once daily. If necessary the dose may be further increased or another antihypertensive agent added. The usual maintenance doses are 5 mg to 10 mg once daily. In elderly patients initial treatment with 2.5 mg daily should be considered.

Stable angina pectoris
The dose should be adjusted individually. Treatment should be started with 5 mg once daily, increasing to 10 mg once daily if needed.

Elderly patients
Initial treatment with 2.5mg daily dose should be considered.

Impaired renal function
Impaired renal function does not affect plasma concentrations of felodipine. No dose adjustment is required. However, Plendil should be used with caution in patients with severely impaired renal function (see Pharmacokinetic Properties).

Impaired hepatic function
Patients with impaired hepatic function may have elevated plasma concentrations of felodipine and may respond to treatment at lower doses (see Special Warnings & Precautions for Use).

Paediatric patients
Due to limited clinical experience use of felodipine to paediatric patients with hypertension should be avoided (see Pharmacodynamic & Pharmacokinetic Properties).

Contraindications
Pregnancy
Known hypersensitivity to felodipine or any other component of the product
Uncompensated heart failure
Acute myocardial infarction
Unstable angina pectoris
Haemodynamically significant cardiac valvular obstruction
Dynamic cardiac outflow obstruction

Special warnings and precautions for use
Felodipine can, like other vasodilators, cause hypotension. This may, in susceptible patients, result in myocardial ischemia.

Mild gingival enlargement has been reported in patients with pronounced gingivitis/periodontitis. The enlargement can be avoided or reversed by careful dental hygiene.

Felodipine is eliminated by the liver. Therefore, higher therapeutic concentrations and a higher treatment response may be expected in patients with clearly impaired hepatic function.

Plendil contains lactose and should not be given to patients with hereditary galactose intolerance or glucose-galactose malabsorption.

Interactions
Felodipine is metabolised in the liver by cytochrome P450 3A4 (CYP3A4). Concomitant administration of substances which interfere with the CYP3A4 enzyme system may affect plasma concentrations of felodipine.

Interactions leading to increased plasma concentration of felodipine.

Enzyme inhibitors of the cytochrome P450 3A4 system have been shown to cause an increase in felodipine plasma concentrations.

Examples:
- Cimetidine
- Erythromycin
- Itraconazole
- Ketoconazole
- Certain flavonoids present in grapefruit juice

Interactions leading to decreased plasma concentration of felodipine.
Enzyme inducers of the cytochrome P450 3A4 system may cause a decrease in plasma concentrations of felodipine.

Examples:
- Phenytoin
- Carbamazepine
- Rifampicin
- Barbiturates
- Hypericum perforatum (Saint John's wort)

Felodipine may increase the concentration of tacrolimus. When used together, the tacrolimus serum concentration should be monitored and the tacrolimus dose may need to be adjusted.

Felodipine does not affect plasma concentrations of cyclosporin

Fertility, Pregnancy and lactation
Fertility
Data on male and female fertility in patients are missing (see Preclinical Safety Data).

Pregnancy
Felodipine should not be given during pregnancy.

Lactation
Felodipine is detected in breast milk. When taken in therapeutic doses by the nursing mother it is, however, not likely to affect the infant.

Effects on ability to drive and use machines
Felodipine is not likely to affect the ability to drive or use machines.

Undesirable effects
Felodipine can cause flushing, headache, palpitations, dizziness and fatigue. Most of these reactions are dose-dependent and appear at the start of treatment or after a dose increase. Should such reactions occur, they are usually transient and diminish with time.

Dose-dependent ankle swelling can occur in patients treated with felodipine. This results from precapillary vasodilatation and is not related to any generalised fluid retention.

Mild gingival enlargement has been reported in patients with pronounced gingivitis/periodontitis. The enlargement can be avoided or reversed by careful dental hygiene.

The adverse drug reactions listed below have been identified from clinical trials and from Post Marketing Surveillance.

The following definitions of frequencies are used:

Very common	≥ 1/10
Common	≥ 1/100 and < 1/10
Uncommon	≥ 1/1000 and < 1/100
Rare	≥ 1/10000 and < 1/1000
Very rare	< 1/10000

Table 1 Undesirable effects

System Organ class	Frequency	Adverse Drug Reaction
Nervous system disorders	Common	Headache
	Uncommon	Dizziness, paraesthesiae
Cardiac disorders	Uncommon	Tachycardia, palpitations
Vascular disorders	Common	Flush
	Uncommon	Hypotension
	Rare	Syncope
Gastrointestinal disorders	Uncommon	Nausea, abdominal pain
	Rare	Vomiting
	Very rare	Gingival hyperplasia, gingivitis
Hepatobiliary disorders	Very rare	Increased liver enzymes
Skin and subcutaneous tissue disorders	Uncommon	Rash, pruritus
	Rare	Urticaria
	Very rare	Photosensitivity reactions, leucocytoclastic vasculitis
Musculoskeletal and connective tissue disorders	Rare	Arthralgia, myalgia
Renal and urinary disorders	Very rare	Pollakisuria
Reproductive system and breast disorders	Rare	Impotence/ sexual dysfunction
General disorders and administration site conditions	Very common	Peripheral oedema
	Uncommon	Fatigue
	Very rare	Hypersensitivity reactions, e.g. angiooedema, fever

Overdose
Symptoms
Overdosage may cause excessive peripheral vasodilatation with marked hypotension and sometimes bradycardia.

Management
Activated charcoal, if necessary gastric lavage.

If severe hypotension occurs, symptomatic treatment should be instituted.

The patient should be placed supine with the legs elevated. In case of accompanying bradycardia, atropine 0.5-1 mg should be administered intravenously. If this is not sufficient, plasma volume should be increased by infusion of e.g. glucose, saline, or dextran. Sympathomimetic drugs with predominant effect on the α_1-adrenoceptor may be given if the above-mentioned measures are insufficient.

Pharmacodynamic properties
ATC code: C08C A02

Felodipine is a vascular selective calcium antagonist which lowers arterial blood pressure by decreasing systemic vascular resistance. Due to the high degree of selectivity for smooth muscle in the arterioles, felodipine in therapeutic doses has no direct effect on cardiac contractility or conduction. Because there is no effect on venous smooth muscle or adrenergic vasomotor control, felodipine is not associated with orthostatic hypotension.

Felodipine possesses a mild natriuretic/diuretic effect and fluid retention does not occur.

Felodipine is effective in all grades of hypertension. It can be used as monotherapy or in combination with other antihypertensive drugs, e.g. ß-adrenoceptor blockers, diuretics or ACE-inhibitors, in order to achieve an increased antihypertensive effect. Felodipine reduces both systolic and diastolic blood pressure and can be used in isolated systolic hypertension.

Felodipine maintains its antihypertensive effect during concomitant therapy with non-steroidal anti-inflammatory drugs (NSAID).

Felodipine has anti-anginal and anti-ischaemic effects due to improved myocardial oxygen supply/demand balance. Coronary vascular resistance is decreased and coronary blood flow and myocardial oxygen supply are increased by felodipine due to dilatation of both epicardial arteries and arterioles. Felodipine effectively counteracts coronary vasospasm. The reduction in systemic blood pressure caused by felodipine leads to decreased left ventricular afterload and myocardial oxygen demand.

Felodipine improves exercise tolerance and reduces anginal attacks in patients with stable effort-induced angina pectoris. Both symptomatic and silent myocardial ischaemia are reduced by felodipine in patients with vasospastic angina. Felodipine can be used as monotherapy or in combination with ß-adrenoceptor blockers in patients with stable angina pectoris.

Felodipine is effective and well tolerated in adult patients irrespective of age and race and is also well tolerated in the presence of concomitant diseases such as congestive heart failure, asthma and other obstructive pulmonary disease, impaired renal function, diabetes mellitus, gout, hyperlipidaemia, Raynaud´s disease and in renal transplant recipients. Felodipine has no effect on blood glucose levels or lipid profile.

Site and mechanism of action
The predominant pharmacodynamic feature of felodipine is its pronounced vascular vs myocardial selectivity.
Myogenically active smooth muscles in arterial resistance vessels are particularly sensitive to felodipine. Felodipine inhibits electrical and contractile activity of vascular smooth muscle cells via an effect on the calcium channels in cell membranes.

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Haemodynamic effects
The primary haemodynamic effect of felodipine is a reduction of total peripheral vascular resistance which leads to a decrease in blood pressure. These effects are dose-dependent. Generally, a reduction in blood pressure is evident two hours after the first oral dose and lasts for at least 24 hours and the trough/peak ratio is usually well above 50%.

Plasma concentrations of felodipine are positively correlated to the decrease in total peripheral resistance and blood pressure.

Cardiac effects
Felodipine in therapeutic doses has no effect on cardiac contractility or atrioventricular conduction or refractoriness. In patients with heart failure, felodipine favourably affects left ventricular function, as assessed by ejection fraction or stroke volume, and does not cause neurohormonal activation. However, felodipine does not seem to affect survival. In patients with hypertension or angina pectoris, felodipine can be used also in case of impaired left ventricular function.

Antihypertensive treatment with felodipine is associated with significant regression of pre-existing left ventricular hypertrophy.

Renal effects
Felodipine has a natriuretic and diuretic effect due to reduced tubular reabsorption of filtered sodium. This counteracts the salt and water retention observed with other vasodilators. Felodipine does not affect daily potassium excretion. The renal vascular resistance is decreased by felodipine. Normal glomerular filtration rate is unchanged. In patients with impaired renal function, the glomerular filtration rate may increase. Felodipine does not influence urinary albumin excretion.

In cyclosporin-treated renal transplant recipients, felodipine reduces blood pressure and improves both the renal blood flow and the glomerular filtration rate. Felodipine may also improve early renal graft function.

Mortality/morbidity data
In the HOT (Hypertension Optimal Treatment) study, the effect on major cardiovascular events (i.e. acute myocardial infarction, stroke and cardiovascular death) was studied in relation to diastolic blood pressure targets ≤ 90 mmHg, ≤ 85 mmHg and ≤ 80 mmHg and achieved blood pressure, with felodipine as baseline therapy.

A total of 18.790 hypertensive patients (DBP 100 – 115 mmHg), aged 50 – 80 years were followed for a mean period of 3.8 years (range 3.3 – 4.9). Felodipine was given as monotherapy or in combination with a betablocker, and/or an ACE-inhibitor and/or a diuretic. The study showed benefits of lowering SBP and DBP down to 139 and 83 mmHg, respectively. When the baseline DBP was lowered from 105 mmHg to 83 mmHg, it suggests that from five to ten major cardiovascular events can be prevented in every 1000 patients treated for 1 year. This implies a 30% risk reduction. Active lowering of blood pressure was particularly beneficial in the subgroup of patients with diabetes mellitus. According to the Swedish Trial in Old Patients with Hypertension-2 study, performed in 6614 patients, aged 70 – 84 years, dihydropyridine calcium antagonists (felodipine and isradipine) have shown the same preventive effect on cardiovascular mortality and morbidity as other commonly used classes of antihypertensive drugs – ACE inhibitors, beta-blockers and diuretics.

Paediatric population
There is limited clinical experience of the use of felodipine in hypertensive paediatric patients. In a randomised, double-blind, parallel group study, the antihypertensive effect of once daily felodipine in children aged 6 – 16 years with primary hypertension was studied. They were treated for three weeks with 2.5 mg (n=33), 5 mg (n=33), 10 mg (n=31) or placebo (n=35).
The study failed to demonstrate any antihypertensive effect in children aged 6 – 16 years.

The long-term effects of felodipine on growth, puberty and general development have not been studied. The long-term efficacy of antihypertensive therapy as therapy in childhood to reduce cardiovascular morbidity and mortality in adulthood has also not been established.

Pharmacokinetic properties

Absorption and distribution
Felodipine is administered as extended-release tablets, from which it is completely absorbed in the gastrointestinal tract. The systemic availability of felodipine is approximately 15% and is independent of dose in the therapeutic dose range. The plasma protein binding of felodipine is approximately 99%. It is bound predominantly to the albumin fraction.
The extended-release tablets produce a prolonged absorption phase of felodipine. This results in even felodipine plasma concentrations within the therapeutic range for 24 hours. Plasma concentrations are directly proportional to dose within the therapeutic dose range 2.5 – 10 mg.

Metabolism and elimination
There is no significant accumulation during long-term treatment.

Average clearance is 1200 ml/min. Reduced clearance in elderly patients and patients with impaired liver function leads to higher plasma concentrations of felodipine. However, age can only partly explain the interindividual variations in plasma concentrations.
Felodipine is metabolised in the liver by cytochrome P450 3A4 and none of the identified metabolites has a vasodilating effect.

About 70% of a given dose is excreted as metabolites in the urine and the rest is excreted in the faeces.
Less than 0.5% of a given dose is excreted unchanged in the urine.

Impaired renal function does not affect plasma concentrations of felodipine, although there is accumulation of inactive metabolites. Felodipine is not eliminated by haemodialysis.

Paediatric population
In a single-dose (felodipine prolonged-release tablet 5 mg) pharmacokinetic study with a limited number of children aged between 6 and 16 years (n=12) there was no apparent relationship between the age and AUC, C_{max} or half-life of felodipine.

Preclinical safety data

Reproduction toxicity
In a study on fertility and general reproductive performance in rats treated with felodipine, a prolongation of parturition resulting in difficult labour/increased foetal deaths and early postnatal deaths was observed in the medium and high dose groups. These effects were attributed to the inhibitory effect of felodipine in high doses on uterine contractility. No disturbances of fertility were observed when doses within the therapeutic range were given to rats.

Reproduction studies in rabbits have shown a dose-related reversible enlargement of the mammary glands of the parent animals and dose-related digital anomalies in the foetuses. The anomalies in the foetuses were induced when felodipine was administered during early foetal development (before day 15 of pregnancy).

List of excipients
Carnauba wax, hydroxypropylcellulose, hypromellose, iron oxides E 172, lactose anhydrous, microcrystalline cellulose, polyethylene glycol 6000, polyoxyl 40 hydrogenated castor oil, propyl gallate, sodium aluminium silicate, sodium stearyl fumarate, titanium dioxide E 171, water purified.

Shelf life
Please refer to expiry date on the label and outer carton.

Special precautions for storage
Do not store above 30 °C.

Pack size
30 tablets in PVC/PVDC blisters

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